NURSING DEPARTMENT

Policy Name : Bathing an Infant Inside the Incubator

Policy Number : BAIITI – NICU 4/3 (8)

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1. **Purpose**
    a. To establish a guideline in giving bath to an infant inside an incubator that will provide and maintain normal body temperature while promoting the well-being of the infant
2. **Scope**
    a. **First Step**
        i. Assess the need of giving bath to the infant
    b. **Last Step**
        i. Documentation of response to the procedure.
3. **Definition**
    a. **Baiting of an infant** - Bathing of an infant is defined as cleaning of the body for the purposes of relaxation cleanliness and healing.
    b. **Umbilical cord** - is the connecting cord from the developing embryo or fetus to the placenta that supplies oxygen, nutrients and a means of waste elimination while the fetus is inside the womb.
4. **Abbreviation:**
    a. RN - Registered Nurse
    b. IV - lnt6ravenous
    c. MRN - Medical Record Number
    d. OGT - Orogastric tube
5. **Policy Statements**
    a. Infants with the following clinical conditions must be given bath inside the incubator:
        i. With cardiac monitor
        ii. With oxygen and IV therapy
        iii. Preterm infant with a weight of less than 1.800 kilograms
        iv. Post operatively patients
        v. IV sites, wound sites or any break in the skin must not be immersed or soaked in water
    b. Standard precautions are to be followed during the procedure.
    c. Proper identification should commence prior to bath.
        i. Name of patient
        ii. Medical Record Number
    d. Baseline vital signs should be taken and recorded.
        i. Infants requiring close observation and monitoring, infants with metabolic disorders.
        ii. Infants below 37 weeks of gestational age.
        iii. Infants with life threatening surgery.

iv. High risk infants requiring extensive sophisticated equipment.

6. **Procedure and Rationale:**
   a. Material/Equipment
      i. Basin or infant tub
      ii. Warm water 37'C
      iii. Mild shampoo or soap
      iv. Towel
      v. Infant clothing
      vi. Diapers
      vii. Soft sponge or cotton
      viii. Pre-warmed linens

Procedure	Rationale
Preparatory Phase	
1. Take the vital signs.	To assess the need of giving bath to the infant and to prevent complications of overcooling.
2. Check the incubator temperature.	Maintains and ensures the appropriate incubator temperature.
Performance Phase	
1. Wash hand and wear gloves.	Promotes hygiene.
2. Prepare all the needed materials.	To facilitate the completion of task in a timely manner.
3. Perform hand hygiene, organize all equipment, and wear gloves.	Reduces microorganism transfer; promotes efficiency of the procedure.
4. Remove OGT, Cardiac leads if any.	To facilitate efficiency of the task and avoid electrical risk.
5. Wet a soft sponge or cotton. Start wiping the eyes and the face taking observation of any eye discharge.	Avoids introduction of microorganism to the eyes.
6. With another wet cotton, apply a mild soap to the scalp in a circular motion taking care not to introduce soap bubbles to the eyes.	Circular motion promotes circulation. Soap bubbles will irritate the eyes.
7. Rinse the scalp with warm water.	Removes soap residue on the scalp.
8. With another wet cotton, gently apply mild soap to the skin taking particular areas on the folds of the skin (neck, axilla and femoral areas). Advance to the genitals and lower limbs.	Some skin debris and dirt can be hidden in the skin fold.
9. Observe for any rashes, redness and skin breakdown.	Proper inspection of the skin aids in early identification of skin integrity problems.
10. Wet another sponge or cotton in warm water and rinse the skin and skin folds.	Removes soap residue from the skin. Retards growth of bacteria and infection.

11. Gently pat dry the infant	Prevents chilling and promotes warmth.
12. Clean the umbilical cord with betadine. Inspect for any signs of foul odor, bleeding or discharge.	Cleanses the umbilical cord. Decrease growth of microorganism and promotes the healing process.
13. Remove all wet linens and replace with clean and warmed linens.	Eliminates cold exposure. Promotes comfort. Reduces microorganism growth.
14. Place the infant in a comfortable position.	Promotes rest and sleep.
15. Document the infant response to the procedure.	Indicates possible changes in client's Response.

7. **Responsibilities**
    a. Clinical Instructor - Responsible for conducting an in-service program to ensure that all staff are adequately knowledgeable.
    b. Head Nurse - Responsible in conducting the unit specific competency check offs and ensures that all staff has satisfactorily completed the competency.
    c. Charge Nurse - Responsible in supervising the actual performance of the staff nurse. Acts a resource person and ensures that all staff satisfactorily completes the check-off every two years.
    d. Staff Nurse - Responsible in attending an in-service education conducted by the resource person. The staff should be competent enough to perform the procedure as per policy statement. The staff should follow the steps as instructed in policies.
8. **Documentation**
    a. Nurses Flow Chart (FM-051)
9. **Measurements**
    a. 100% of the nurses should complete the competency check off after three months of probationary.
10. **References**
    a. The Lippincott Manual of Nursing Practice, 9th edition
    b. Nurse's Guide to Clinical Procedures, 5th edition
    c. Procedure Manual for Pediatric Acute and Critical Care, Saunders
11. **Cross References**
    a. CBAHI Standards
    b. Policies Procedures and Guidelines, Third Edition
12. **Attachments**
    a. Narcotic form
    b. Expiry dates of the drugs check list.
13. **Distribution List**
    a. Nursing Department
    b. Quality Department
    c. Medical Administration
    d. HOD of pediatric dept.

e. Executive Director General

14. **Review Summary**
    a. Additional-definition